1. Caesarean section and rheumatoid arthritis

Setting: maternity ward

Patient: you are in hospital for delivery for your third child by caesarean section, when you are also diagnosed with rheumatoid arthritis. You have severe pain in your wrists and shoulders and you are feeling very fatigued, as the pain is disturbing your sleep. You are worried about using any medication while breastfeeding.

Task: 1. complain to the nurse about your pain but insist on not taking any oral medications for the pain or sleeping problems.
2. Express your fear that the medication may harm your baby and you are determined to breastfeed the baby
3. Firmly insist on using a natural gel product which contains capsaicin and is reputed to help with pain relief. Be very agitated by the nurse's insistence on prescribed medication.
4. Ask how soon you should expect to be able to go home.

Nurse: this 41 year-old patient is in hospital for delivery of her third child by caesarean section, when she is also diagnosed with rheumatoid arthritis. She has severe pain in her wrists and shoulders and is feeling very fatigued, as the pain is disturbing her sleep. She is worried about breastfeeding whilst using any medications.

Task: 1. respond sympathetically to the patient's demands and questions.
2. Explain the reasons for the oral medication prescribed by her/ his doctor and try to persuade the patient to take this medication.
3. Reassure the patient of the safety of the oral medication. Try to calm the patient down by suggestion other ways to also cope with the pain (hot/cold pack, arrange for friend/ relative support, review by doctor).

N: I see. I understand what you thought about. I am sorry to hear that. (1) However, your doctor has prescribed oral medications for that. Did you take them? /Could you please tell me why you did not take them? These medications are very effective to slow the progress of the disease, control your symptoms, and reduce pain. They have been used in clinical practice for many years. There are no unpleasant side effects on your breastfeeding. So don't worry about breastfeeding while using this medication (2). Apart from oral tablets, there are other forms of medications, such as gel and cream. They are used on the topical areas where they are needed. They are not dependent on circulation through bloodstream. So they are safe to be used during your breastfeeding. (3)
With regard to your sleeping problems, we have some other type of treatments which may be equally effective as taking medications; we will arrange a warm bath and then offer you a glass of warm milk for you before you go to bed tonight. We will see if they work.
Now the important problem is that you are in so much pain. I will let your doctor know. He will probably review you soon. And I will get you a hot pack. The warm compress will be helpful for you to relieve pain. Have you got any family or friends to help you? If I contact them and arrange them to visit you or stay with you for a while, would you feel better? Generally, a week after the caesarean section you can go back home. But it also depends. When your wounds heal well, there are no complications and your pain is controlled very well, you will be able to go home.

2. UTI and husband

Setting: hospital ward

Patient: as a 72 year-old patient who have developed a urinary tract infection after underwent surgery 4 days ago for Colon cancer. You were commenced on an antibiotic and Ural. You live with your husband, who has chronic obstructive airways disease and is partially blind and very reliant on you. Despite your condition, he is your biggest concern as he is currently home alone.

Task: 1. ask questions about how long you need to take medication, how long you will need to stay in hospital, and whether chemotherapy will be dangerous at your age.
2. Answer the nurse's questions and explain that you had an episode of unconsciousness and were not feeling well for a few months, lacking energy and tiring quickly.
3. Express concern about staying in hospital as your husband is home alone and relies on you for help. Be very worried about this.

Nurse: this 72 year-old patient has developed a urinary tract infection after he / she underwent surgery 4 days ago for cancer of the colon. She was commenced on an antibiotic and Ural. The patient lives with his wife/ her husband, who has chronic obstructive airways disease and is partially blind and very reliant on you. He / she is concerned about a prolonged stay as her husband / his wife is home alone.

Task: 1. answer the patient's questions in a reassuring manner.
2. Ask the patient about his / her general health over the last six months and how he/she has been coping at home.
3. Sympathies with the patient and try to calm them down by offering home help for the care of the patients\'s husband / wife. Give advice about diet and lifestyle to assist the patient to cope better.

N: Hello. Mrs. Smith, good afternoon. I am XX. I am a registered nurse. I will look after you this afternoon. How are you feeling now?

P: I feel worried. I have got a urinary tract infection and now I begin to take antibiotics. I wonder how long I need to take them and how long I will need to stay in hospital. And whether chemotherapy will be dangerous at my age, you know, I am 72 year old.

N: No worries. Urinary tract infection is a common complication after a surgery. And your situation is not serious. You begin to take antibiotics and they will control the infection well .you need to take the medication for a course, about 7 days. So you will stay in hospital for the next 7 days, then we will test your urine again. If the result is negative, you will be able to go home. The chemotherapy is much easier to tolerate today even a few years ago. It is safe for people at your age. Although there are some side effects of chemotherapy, such as nausea, hair loss, fatigue, but it is quite individual and your doctors and nurses will keep close track of side effects and can treat most of them to improve the way you feel.

Did your doctor tell you about your diagnosis? It is colon cancer. I am sorry about that .I would like to know how you feel about the last six months.

P: I had an episode of unconsciousness and I was not feeling well for a few months, lacking of energy and be tired quickly.
N: I am sorry to hear about that. What have you done?

P: I just lay on my bed and had a rest.
N: I see. Colon cancer may be resulted from an unhealthy diet, drinking, smoking and lack of exercise. So would you mind if I ask you some questions? Ok, How about your diet? What kinds of food do you like to eat? Do you eat fruits and vegetables every day? Do you smoke / drink? Do you do exercise?

P: I like fast food, meat. I do not eat fruits and vegetables every day. I smoke and drink. I do not exercise.
N: I see. You need a healthy diet. You need to eat more vegetables, fruits and less meat. Dies with too much meat is high in fat and cholesterol, have been linked to increased colon cancer risk. High-fiber food is healthy, such as whole meal bread, fruits and green vegetables, which have shown protective effects. Besides, I suggest you to give up smoking and drinking, because they may transport carcinogens to the colon, so they increase your risk of colon cancer. In addition, a sedentary lifestyle can lead to colon cancer development. So I recommend that you should do exercise regularly. For example, you may have a walk around your house for 5 to 10 minutes everyday.

P: I am worried about my husband. I live with him, who has chronic obstructive airways disease and is partially blind and very reliant on me. Now I am in the hospital. He is at home alone. I am worried about him.
N: I see. I understand. No worries. I will give you some help. For example, meals on wheals. They offer three meals so that your husband does not have to cook by himself. And I will assign a carer to help him with housework, such as dressing, bathing, washing, cleaning, gardening, and things like that. The carer also can do shopping for him and chat with him. I will give you the numbers of those support services. If you need anything else once you have been discharged, you can put services in place. These services also can help you a lot and you will have more time to rest.
Remember, you need rest for full recovery. The most important thing is to remember that we are always here ready for help. After you go back home, we will also keep an eye on you. You need a regular checkup. Remember, if you feel unwell, you need to see your GP. I suggest you to see your GP more often. I have some booklets about colon cancer, if you know a little more about what it is, what to expect and what help is available, you will not feel so worried, I hope. If you are interested in them, I will bring them to you. Do you have any other questions?
P: No, thanks.

N: all right. Now I leave you to do something else. I will be back soon with those booklets and I will tell you what services I arrange for your husband .is it ok? Here is the bell, please do not hesitate to give a buzz if you need me. Remember we are always here for help. See you later.

Chemotherapy is a systemic therapy; this means it affects the whole body by going through the bloodstream. The purpose of chemotherapy and other systemic treatments is to get rid of any cancer cells that may have spread from where the cancer started to another part of the body.

Chemotherapy is effective against cancer cells because the drugs love to interfere with rapidly dividing cells. The side effects of chemotherapy come about because cancer cells aren't the only rapidly dividing cells in your body. The cells in your blood, mouth, intestinal tract, nose, nails, vagina, and hair are also undergoing constant, rapid division. This means that the chemotherapy is going to affect them, too.

Still, chemotherapy is much easier to tolerate today than even a few years ago, and for many women it's an important "insurance policy" against cancer recurrence. It's also important to remember that organs, in which the cells do not divide rapidly, such as the liver and kidneys, are rarely affected by chemotherapy, and doctors and nurses will keep close track of side effects and can treat most of them to improve the way you feel.

In this section, you'll learn more about how chemotherapy works and the best ways to manage nausea, hair loss, and other chemotherapy side effects.

You'll also read about the different chemotherapy regimens and about deciding together with your medical oncologist which regimen would be best suited to you.

It's important to remember that every woman's ideal treatment plan is different. Be aware that your "chemo" regimen may be different from someone else's, based on very individual—and sometimes subtle—breast cancer factors. These include: lymph node involvement, tumor size, hormone receptor status, grade, and ontogeny expression. Be prepared for your doctor to recommend a combination of chemotherapies—together or in a series.

Causes of Colon cancer: age / alcohol / smoking/ diabetes / diets / environment / ethnicity, race and social status / family medical history/ gender / genetics/ inflammatory bowel disease (IBD)/ lack of exercise/ obesity/ personal medical history/ polyps.

Colon cancer prevention tips:

1. Go to see a doctor if you have any colon cancer symptoms(thin stools, cramping, unexplained weight loss, bloody stools)
2. if you're 50 or older, schedule a colon cancer screening (excellent colon cancer prevention)
3. eat a balanced diet
4. maintain a healthy weight
5. maintain an active lifestyle (exercise)
6. consider genetic counseling (FAP/ HNPCC)
7. learn your family medical history
8. talk to a doctor about your personal medical history
9. don't smoke
10. Reduce radiation exposure.

3. Epilepsy

Setting: day clinic

Patient: you are 52 year-old who has recently returned to Australia after a year away. Whilst overseas, you were involved in a traffic accident and suffered a head injury. Today the doctor has diagnosed you with post-traumatic epilepsy. You have been advised to take medication to help prevent further attacks.

Task: 1. answer the nurse's questions and explain that since your traffic accident you have had a history of recurrent blackouts. Prior to the attacks, you have headaches and afterwards, you feel lethargic and less alert than usual.
2. Ask how long you will need to take medication and what the risks of the medication are. Ask about epilepsy and the implications of epilepsy on a traveling lifestyle.
3. Be concerned about long term use of the suggested medication and be reluctant to accept what the nurse is telling you. You have heard that the medication can prevent travel in some countries that you wish to travel to in the future.

Nurse: this 52 year-old patient has recently returned to Australia after a year away. Whilst overseas, she was involved in a traffic accident and suffered a head injury. Today the doctor has diagnosed her with post-traumatic epilepsy and advised her to take medication to help prevent further attacks. The doctor told the patient

Task: 1. sympathetically ask the patient how she has been feeling since the traffic accident.
2. Answer the patient's questions and reassure her of the safety of the prescribed medication. Explain that because it is usual to continue medication until there have been no seizures for at least 4 years, it is possible a lifelong course of medication.
3. Explain that the headaches and blackouts can be effectively and safely controlled by the medication and enable a person to lead a normal lifestyle.
4. Reassure the patient that her travel plans will not be affected by medication. Offer them some literature about epilepsy and support services available.

N: Hello, Mrs. Smith, good after. I am XX. I am a registered nurse. I will be looking after you this after. Has the doctor told you your diagnosis? It is post-traumatic epilepsy. I have heard that you had a traffic accident and suffered a head injury. I am sorry about that. Could you tell me how you have been feeling since the traffic accident?

P: Since my traffic accident, I have had a history of recurrent blackouts. Before that, I have headaches and afterwards, I feel tired and less alert than usual.
N: I see. I am sorry to hear that. The doctor has prescribed medications for you.

P: How long will I need to take medication and what are the risks of medication?
N: Generally, it is usual to continue medication until there have been no seizures for at least 4 years. But it depends on individuals; it is probable a lifelong course of medication. However, the medication is safe for long term use. It has been used in clinic for many years. Many people in your

situation have taken the medication. There are little unpleasant side effects.
P: Actually, I know little about epilepsy. Are there implications of epilepsy on a traveling lifestyle? I have to travel a lot due to my job.

N: Epilepsy is a condition in which a person has spontaneously recurrent seizures. And seizure is a transient symptom of excessive neuronal activity in brain. Seizures are often associated with a sudden and involuntary contraction of muscles and loss of consciousness. Some conditions could trigger seizures, like excessive alcohol intake, overtiredness or poor quality of sleep. However, don't worry. if you take the medication regularly, have an 8 hour sleep every night, eat regular meals and do not make yourself too tired, your headaches and blackouts would be controlled effectively and safely. The medications enable you to lead a normal life. If there have been no seizures for at least 4 years, you would not need the medication any more. But before stopping the medication, you will need a checkup by your doctor and epilepsy does not impact on your traveling. But I suggest you to wear a medical alert bracelet or necklace or identification card with you.

P: I do not want the medication for long term use. I have heard the medication can prevent travel in some countries that I wish to travel to in the future.

N: I see. I understand why you are feeling this way. But to my best knowledge, there are no countries that have this kind of policy. But I will check it for you. I will tell you some useful websites, so that you could search some information. And I have some booklets about epilepsy, if you know a little more about what it is, what to expect and what help is available, you would not feel so worried, I hope. If you are interested in them, I will bring them for you. Is there anything else you want to ask or discuss? Do you have any other questions? Please ask any question, if I do not know the answer, I will find out for you .now, I leave you to have a rest. Here is the bell, please do not hesitate to give me a buzz if you need me. Remember we are always here for help. I will be back soon with the booklets.

4. Balloon Angioplasty

Setting: hospital ward

Patient: You are 67 years old and have been admitted to hospital for balloon angioplasty. Six months previously, you had a coronary artery bypass graft , form which you made a good recovery. This admission, there is obstruction to the coronary arteries by further atherosclerotic deposits. You have lived on your own since your husband's death 3 years ago.

Task: 1. Answer the nurse's questions and explain that you get most of your nutrition from take away food. You have cut down on cigarettes from 20 to 10 a day. You do not exercise out of fear of further heart problems. You have a son who lives on the other side of town.
2. Tell the nurse you fear for your health and ask for advice about lifestyle improvements.

Nurse: a 67 year-old patient is admitted to hospital for balloon angioplasty. Six months ago she had a coronary artery bypass graft, form which she made a good recovery. Now there is

obstruction to the coronary arteries by further atherosclerotic deposits. Her current medical history indicates that she may need to look at lifestyle changes. She has lived on their own since his husband's death 3 years ago.

Task: 1. ask the patient about her diet, exercise, smoking and drinking habits and any support networks of family and friends.
2. Reassure the patient the lifestyle changes can be made easily. Emphasis the importance of moderate exercise and eating healthier foods.
3. Discuss exercise options such as waling or lawn bowls and setting achievable goals for dietary improvements (e.g., start by cooking once a week, avoid unhealthy foods when grocery shopping).

N: Hello, Mrs. Smith, good afternoon. I am a registered nurse. I will be looking after you this afternoon. Your test has shown there is obstruction to your coronary arteries so that you need a balloon angioplasty, and I know six months ago you had a coronary artery bypass graft, followed by a full recovery. The clots are caused by high levels of blood fat. So I would like to ask you some questions about your lifestyle. What do you usually eat? Do you smoke? How many cigarettes do you smoke every day? Do you drink Wine, beer or alcohol? Do you doing exercise every day? Do you live alone? Have you got any family or friend to help you?

P: I get most of my nutrition form take away food. I dislike cooking. I am a smoker but I have cut down on cigarettes from 20 to 10 a day. I drink a glass of alcohol every night. I do not exercise out of fear of further heart problems. I have lived on my own since my husband's death 3 years ago. I have a son who lives on the other side of town.

N: Most take-away food contains high levels of sugar, salt and fat. They are not healthy food. They will increase your blood fat levels. I suggest you to cook more healthy food for yourself. If you really don't like cooking. I could arrange meals on wheals for you. They offer healthy food. Healthy food is those full of fibres, like whole meal bread, fruits and vegetables, especially green ones. Not too much meat with less salt, sugar and fat.
I'm sure you know the damage of smoking, so you have cut down on cigarettes from 20 to 10 a day. Congratulation! I know giving up smoking is very difficult but I did it. I recommend you to continue till you give up smoking completely. Thus, alcohol is high in calories and has no nutritive value as well as it would damage the liver and blood vessels, so I suggest you to quit drinking or a glass of wine is better than alcohol.

I see. Most people in your same situation feel the same way. Exercise is very important. Even a small amount of exercise every day will benefit you. Even you do exercise when you are watching TV. Exercise would reduce blood fat, make your blood pressure in good levels, and make your muscles stronger, including the muscles in your blood vessel walls. I could have a walk around your house for 5 to 10 minutes every day or join in a lawn bowls club that will help a lot. I know you are afraid of further heart problems due to exercise. no worries. If you fear so much, I would refer you to a physical therapist, who will design a special exercise plan according to your own situation and give you special instructions. Sounds good? If you join in a club, I would make new friends and have fun and get support from them.
I have some booklets about coronary artery disease, if you know a little more about what it is, what

to expect and what help is available, you would not feel so worried I hope. If you are interested in them, I will bring them for you. do you have any other questions? Please ask any question, if I do not know the answer, I will find out for you. Now I leave you to have a rest. Here is the bell. If you need me, please do not hesitate to give me a buzz. Remember we are always here for help. I will be back soon with those booklets. See you later.

5. Hyperemesis gravidarum

Setting: hospital ward

Patient: You are 29 year-0ld nullipara woman suffering from persistent vomiting and dehydration. You have been diagnosed with hyperemesis gravidarum and admitted to hospital. You have commenced on IV therapy, complete bed rest, and IV pyridoxine. Ongoing treatment includes an oral anti-emetic to continue until the end of the second trimester. Your husband is in the waiting room. (Nullipara: a woman who has not given birth to a viable infant)

Task: 1.ask the nurse to explain what hyperemesis gravidarum is and what the implications are for the baby.
2. display anxiety and persist with questions about your medication. You have not taken any medication to date as you fear harm to the baby.
3. further express anxiety about losing this baby.

Nurse: this 29 year-old nullipara woman is diagnosed with hyperemesis gravidaum and admitted to hospital. She is commenced on IV therapy, complete bed rest, and IV pyridoxine. Ongoing treatment includes an oral anti-emetic to continue until the end of the second trimester.

Task: 1.Answer the patient's questions. Explain that hyperemesis gravidarum is an abnormal condition of pregnancy associated with prolonged vomiting, weight loss and fluid and electrolyte imbalance. Her case is not severe and the doctor expects she will recover fully and deliver a healthy baby.
2. Reassure the patient that the medication will not harm her baby. Ask if she would like to see her husband for some emotional support.
3. Ensure all the patient's concerns are dealt with.

N: Hello, Mrs. Smith. Good afternoon. I am a registered nurse. I will be looking after you this afternoon. How are you feeling now?
P: I am worried. The doctor said my diagnosis is hyperemesis gravidarum. I know little about it. Could you tell me something about it?
N: yes. It is a common disease when a woman is pregnant. The main symptoms include vomiting, weight loss and fluid and electrolyte imbalance during the first 6 months of the pregnancy. If vomiting is too heavy, it would impact on baby's development but no worries. Your situation is not serious. Most women during pregnancy have the same problem. You need complete bed rest and the doctor has ordered an oral anti-emetic and IV therapy for you. You need to take the anti–emetic

till the end of second trimester. The doctor expects you will recover fully and deliver a healthy baby.

P: I know the doctor has prescribed some medication for me. What is it for? I have not taken any medication to date as I fear harm to the baby.
N: I see. I can understand why you are thinking this way. Especially, it is your first time to be pregnant. Many women in your same situation feel the same way. They don't want to take any medication. They don't want to harm their babies but don't worry. The medication is actually vitamin b6. It will relieve your vomiting. It is effective and safe for both you and your baby. Many women with the same problem have taken the medication. It has been used in clinic for many years and there are no unpleasant side effects. They all have delivered healthy babies. The IV therapy is just to provide plenty of fluid for you. They will benefit you and your baby. If you don't vomit any more, you will drink and eat more and you will have more nutrition for yourself and your baby. If I arrange your husband to come here and stay with you, will you feel better?
I have some booklets about the disease, if you know a little more about what it is, what to expect, I hope you would not feel so worried. If you are interested in them, I will bring them for you. Do you have any other questions? Please ask any question, if I don't know the answer, I will find out for you. Now I leave you to have a rest. Here is the bell. If you need me, please don't hesitate to give me a buzz. Remember we are always here for help. I will contact your husband and I will be back soon with the booklets. OK?

6. Pulmonary clots

Setting: intensive care unit

Patient: you are a 67 year-old, hospitalized for exacerbation of chronic obstructive airways disease. You are worried and anxious about your husband who collapsed while visiting you. He complained of sudden chest pain and then fainted. He had mentioned he had calf pain in the left leg over the previous week. A pulmonary angiogram revealed a large clot, and so an urgent embolectomy was performed. Afterwards, he was transferred to the intensive care unit in a stable condition.

Task: 1. Be very anxious. Ask why your husband had to have an operation.
2. Ask the nurse what could have caused the clot. Is this normal for a man in their sixties?
3. Ask how the risk of clots can be minimized.
4. Ask how long it will be before your husband can be taken home.

Nurse: The patient is a 67 year-old, hospitalized for exacerbation of chronic obstructive airways disease. She is worried and anxious about her husband who collapsed while visiting her. Her husband complained of sudden chest pain and then fainted. He had mentioned he had calf pain in the left leg over the previous week. A pulmonary angiogram revealed a large clot and so an urgent embolectomy was performed. Afterwards, he was transferred to the intensive care unit in a stable condition.

Task: 1. Reassure the patient and answer her questions.

2. Explain strategies to the patient for prevention of clots (such as not crossing her legs while sitting or lying)
3. Answer the patient's questions and reassure tem that their husband is currently in a stable condition and that when discharged regular blood tests will be advised to ensure no risk of future clots.
4. Explain to the patient what he can look out for in order to avoid this happening again.

N: Hello, Mrs. Smith. Good afternoon. I am a registered nurse. I will be looking after you this afternoon. How are you feeling now?
P: I am so worried about my husband. My husband complained of sudden chest pain and then fainted when he visited me. Then he had an urgent operation. After that he is in the ICU. I don't know why he had to have the operation.
N: No worries. Now your husband is in the ICU. There are skilled, experienced nurses to look after him and he is in a stable condition. The test has shown there was a large clot in the blood vessel of his lung. The clot would cause blood flow to reduce suddenly and then a heart failure. That is why he complained of sudden chest pain and then fainted. This clot is dangerous. It must be removed as soon as possible. That is why he had to have the operation. During the operation, the clot has been removed. Now he is stable.
P: I know little about the disease. Could you tell me what could have cause the clot. Is this normal for a man in his sixties?
N: There are many factors can cause a clot. Such as, age, gender, smoking, drinking, lack of exercise, overweight, an unhealthy diet. Yes, it is a common problem for a man in his sixties. Most elder people have the same problem. Although we can not change our age and gender, there are still many things we can do to prevent clots. Things like, regular exercise, a healthy diet, weight loss, giving up smoking and drinking and not crossing our legs when sitting or lying.

P: I am still worried about my husband. How can the risk of clots be reduced?
N: I see. I can understand why you are feeling this way. No worries. Your husband is in a stable condition. When he is discharged, having regular blood tests will make sure no risk of future clots. He may need some changes in his lifestyle, for example, regular exercise, even a small amount everyday will benefit him or even do exercise when he is watching TV or have a walk around your house for 5 to 10 minutes everyday or join in a club. Does he smoking? Does he drinking? I suggest him to quit smoking and drinking. Our hospital has a special quitting program. I could contact them for him. Having a healthy diet is so important too. It includes plenty of fruits and vegetables, especially green ones, not too much meat with less salt, sugar and fat. Is he overweight? I can refer him to a physical therapist to lose weight.
There are special warning signs before clots block blood vessels. If he complains of pain in his legs or chest or he could not see things clearly, he must contact his GP immediately. I recommend him to see his GP more often and have regular blood tests so that this situation would not happen again.

P: How long will it be before he can be taken home?
N: Generally, he needs to stay for a week but it depends on individuals. If he is in a stale condition without any complications, he will be discharged a week later. Is there anything else you would like to ask and discuss? Please ask any question, if I do not know the answer, I will find out for you. I

have some booklets about clots, if you know a little more about what it is and what to expect, I hope you would not feel so worried. If you are interested in them, I will bring them for you. Now I leave you to have a rest, here is the bell, if you need me, please do not hesitate to give me a buzz. Remember we are always here for help. I will be back soon with the booklets. See you later.

(If blood fat levels are higher than normal, clots would be formed. When clots are big enough, they would block the blood vessels completely and stop the blood flow and cause many troubles.
Many factors can increase blood fat levels. Lack of exercise would slow the blood flow. Overweight means too much fat in the blood. Smoking and drinking would damage the blood vessels and make them narrow and hard. All of them make clots more easily formed.)

7. Tonsillectomy

Setting: pediatrics ward

Patient: you are the mother of Michael, aged ten, who has had a tonsillectomy and is due for discharge the day after surgery. You think Michael should stay longer, as he says he is still in severe pain and does not appear to be very alert. Michael has a fever and looks unwell. He is having difficulty taking fluids. He is on Tylenol for the pain.

Task: 1. express concern to the nurse about your son's pain and request he be kept in hospital longer for observation.
2. Ask for information about giving his medication at home. What dosage should be receive and how often?
3. Show anxiety. If the pain worsens, what should you do? Are there any special precautions you should take?
4. What should he eat and drink?
5. When can friends visit? When can he return to school?

Nurse: a 10 year-old boy named Michael, aged ten, has had a tonsillectomy and is due for discharge the day after surgery. His mother thinks he should stay longer, as she says he is still in severe pain and does not appear to be very alert. He has a fever and looks unwell. He is having difficulty taking fluids. He is on Tylenol for the pain. His recovery is expected to take between 10 and 20 days. The parent appears anxious about the child's condition.

Task: 1. reassure the patient about Michael's pain, explain that a tonsillectomy is a standard procedure and his current level of pain is normal. Reassure the parent that a longer hospital stay is not necessary.
2. Explain to the parent how to administer his medication.
3. Answer the parent's questions.

N: Hello, Mrs. Smith, good afternoon. I am a registered nurse. I will be looking after your son this afternoon. Your son has had the tonsillectomy, tomorrow he will be discharged. So I thought it

might be a good idea to talk about his discharge arrangements so you know how to look after him and things are all in place for him when he goes home. You look worried, don't you?
P: Yes. I am very worried about my son. He has a fever and in serious pain and looks unwell. I think he should stay in the hospital longer for observation.
N: I see. I can understand why you are feeling this way but no worries. A tonsillectomy is just a standard procedure, a small operation. Most children have the operation in their childhood. After the operation, many things are normal. Things like having a pain and having a fever and having difficulty taking fluids. The day after the operation, everything would be fine. Pain would be controlled very well by painkiller, temperature would be normal and swallow would be getting better. So his current level of pain is normal and the doctor has prescribed Tylenol for the pain. It is an effective painkiller for children and there are no unpleasant side effects. Most children in his same situation have taken the painkiller. Every day he will feel a litter better. A longer hospital stay is not necessary.
P: Thanks, now I feel better. He will take his medication at home. I want to know what dosage should be receive and how often.
N: The doctor will write a prescription for any medication for him. We can get the prescription filled for you so you won't have to go to the chemist on the way home. There might be some changes to his medication, so we will write a list of what he needs and when to take it. Generally, he needs 1 tablet for once and three times a day. I will make sure you have the list before he goes home. He will have difficulty taking fluids for the first day or two that he is home, so I suggest you crush up the whole pills for him for the sake of safety.
P: Are there any special precautions I should take? If the pain worsens, what should I do?
N: You need to make sure he takes the painkiller regularly and if the pain worsens, you should contact the doctor immediately. I suggest you to check his temperature every 4 hours. His swallow would be getting better. If it is worse or he has trouble breathing, the wound must be swelling or bleeding, you should contact the doctor immediately. I will arrange the home nurse to visit him. She will check the wound daily for him. Using a humidifier at home can also bring some comfort. Your child may experience alternating good and bad days for the first two weeks after surgery. It is a good idea to keep your child away from crowds or ill people for the next 7 days, since the throat is highly susceptible to infections during this period.
P: What should he eat and drink?
N: Food should be soft and not too hot. Ice cream is better. Cold food and drink would prevent the wound swollen and bleeding. I suggest to eat soft, easy to swallow food and to drink a lot of cold fluids.
P: My son has a lot of classmates and friends. They all worry about him. When can they visit him? And when can he return to school?
N: His recovery would take between 10 and 20 days, generally. After the operation, his immune system is not as good as before, it would take 10 to 20 days to recover and even healthy people carry germs, so I suggest his friends visit him 20 days later. At that time, he could return to school but for the sake of safety, it would be a good idea to see the doctor first.
I have some booklets about tonsillectomy, if you know a little about what it is and what to expect, I hope you would not feel so worried. If you are interested in them, I will bring them for you. Is there any thing else you would like to ask or discuss? Please ask any question, if I do not know the answer, I will find out for you. Now I leave you and your son to have a rest. Here is the bell, if you need me, please do not hesitate to give me a buzz. Remember we are always here for help. I will be back soon with the booklets. See you later.

8. Wound infection

Setting: hospital ward

Patient: You are a 35 year-old patient who had abdominal surgery four days ago for removal of a large benign tumor, severe abdominal pain and vomiting over two days prompted you to have tests done as you are usually in good health. You had been suffering intermittent abdominal pain for about 3 months. Your recovery from the operation has been complicated by a wound infection. Now, you are anxious that not all of the mass was found and you are worrying that the tumor may have been malignant.

Task: 1. ask the nurse whether it's possible that the tumor was malignant. How could she be certain?
2. Do not be easily reassured. Express your concern that not the entire tumor mass was removed.
3. Ask now much longer you will need to stay in hospital. Is it possible the tumor will reappear? When can you recommence your cycling routine?

Nurse: The patient is 35 year-old who had abdominal surgery four days ago for removal of a large benign tumor. Now, in the fourth day after surgery, her recovery has been complicated by a wound infection. The infection is minor but the patient appears anxious.

Task: 1. reassure the patient about the certainty of the diagnostic process for tumors.
2.Try to calm the patient by explaining the infection is not serious. Make some suggestions to make her more comfortable (e.g. more pillows, bed readjustment). Check the patient knows how to readjust her bed.
3. Answer the patient's questions and deal with the patient's anxiety about the tumor.

N: Hello, Mrs. Smith. Good afternoon. I am a registered nurse. I will be looking after you this afternoon. How are you feeling now?
P: I am worrying the tumor, I think it is malignant.
N: No worries. Your tumor is benign. The lab report has shown it is benign. You know, every tumor would be tested after it is removed. And the report has been made by a couple of pathologists to make sure it is correct. I can understand why you are thinking this way. Many people in your same situation think their tumors are malignant till they receive the pathological report. Do not worry, I am sure the report has shown it is benign.

P: My concern is that not the entire tumor mass has been removed.
N: No worries. Your tumor has been removed completely. A benign tumor is different from a malignant one. A benign one is covered by a whole membrane. When a doctor removes a benign tumor, it is very easy to remove it completely and it is a routine for the doctor to check its membrane to make sure all of the tumor mass has been removed. Your doctor is an experienced

doctor and he has done hundreds of this kind of operations every year. Till now, you have made a great recovery except your wound infection but no need to worry. The infection is minor. The doctor has prescribed antibiotics for you. The medication is safe and effective. Your wound infection would be controlled very well and there are no unpleasant side effects and complications. Many people in your same situation have taken the medication. It has been used in clinic for many years. You have got a pain in your wound, have not you? If I put one more pillow behind you, will you feel better, more comfortable? Here you go. Here is the bed control. Let me show you how to use it. The button can make the head of the bed up and this button can make it down. Is it clear? Are you with me so far? Ok, now, show me; let us make the head of the bed up, that's it. So you can make a more comfortable position are you feeling better?

P: How long should I stay in hospital?
N: In general, it takes a week to recover but it depends on individual's situation. In your situation, your wound infection would be controlled very well in 4 or 5 days. So you might need to stay in hospital for the next 7 days.
P: Is it possible the tumor will reappear?
N: Generally. Most benign tumors would reappear after they are removed. They are different from malignant ones but now I can't say for sure. You need a regular checkup and we will also keep an eye on you.
P: when can you recommence my cycling routine?
N: It is one week after your discharge. I have some booklets about abdominal tumors; if you know a little more about it, what to expect, I hope you would not feel so worried. If you are interested in them, I will bring them for you. Is there any thing else you would like to ask or discuss. Please ask any question. If I do not know the answer, I will find out for you. Now I leave you to have a rest. Here is the bell, if you need me, please do not hesitate to give me a buzz. Remember we are always here for help. I will be back soon with the booklets. See you later.

9. Hypertension

Setting: hospital ward

Patient: You are aged 55, and while in hospital for elective surgery on your knee, you have been told you have hypertension. You play tennis intermittently and know you should reduce your cigarette smoking and your weight. As you are a busy business person with a high stress job and travel interstate frequently, you would prefer to manage your health with medication. Your husband is constantly nagging you to drink less beer in order to lose weight and is very supportive of you.

Task: 1. answer the nurse questions and explain that you often eat out at regular places with clients. You play tennis, although not always regularly as you travel frequently for work.
2. Ask what medication you can take to lower your blood pressure.
3. Ask the nurse how you could make effective changes to your lifestyle. Be hesitant a first about the nurse's suggestions, but eventually be willing to compromise.

Nurse: This 55 year-old patient was found to have hypertension while in hospital for elective knee surgery. She is quite obese, and the patient's history reveals she is a heavy smoker and suffers alcohol abuse. Her husband is concerned about her excessive drinking and thinks she should cut down. The patient would prefer to use mediation instead of trying diet and exercise programs because she has little time for such programs due to being busy with a stressful job and frequent travel interstate.

Task: 1. ask the patient about her diet and exercise.

2. Explain to the patient that medication alone would not be effective in this case because her high blood pressure is due to excess calories and saturated fats in her diet, insufficient exercise, heavy smoking and drinking too much alcohol. Strongly advise lifestyle changes.
3. Discuss strategies with the patient for dietary and exercise improvements. Emphasize the need for the patient to dramatically reduce her intake of alcohol and tobacco. Try to reach a compromise with the patient about lifestyle changes.

N: Hello. Mrs. Smith, good afternoon. I am a registered nurse. I will be looking after you this afternoon. By observations, we have found you have hypertension, which is a high level of blood pressure. That means your blood pressure is a little bit higher than normal levels. In most cases, hypertension is caused by an unhealthy lifestyle. So would you like to talk about your diet and exercise? What do you usually eat? And do you do exercise? Do you smoke? How many cigarettes do you smoke every day? Do you drink wine, beer or alcohol?
P: I often eat out at regular places with my clients.
N: What kind of food do you usually order?
P: Meat, in most time. I play tennis intermittently. I travel a lot interstate frequently due to my job. I could not do exercise regularly. I am a heavy smoker and suffer alcohol abuse.
N: OK. Now I have an idea of your lifestyle. As I said, hypertension is caused by an unhealthy lifestyle that includes an unhealthy diet, with much salt, sugar and fat in it. Lack of exercise, smoking, drinking and a lot of stress. In your case, I have to say your hypertension is due to excess calories and saturated fats in your diet, lack of exercise, heavy smoking and drinking too much alcohol and your weight as well. So I suggest you to change your lifestyle. Though the doctor has prescribed medication to control you blood pressure but in you case, medication alone would not be effective.
P: What medication you can take to lower your blood pressure.
N: there are many kinds of medications for blood pressure control. Say diuretics, adrenergic inhibitors, direct vasodilators, ace inhibitors, angiotensin-concerting enzyme inhibitors and calcium channel blockers. Before you are discharged, the doctor will write a prescription for any medication for you. We can get the prescription filled for you so you would not have to go to the chemist on the way home and there might be some changes for your medication, we will write a list of what you need and when to take it. I will make sure you have the list before you go home but as I said, in you situation. Medication alone would not be effective. You need to change your lifestyle.

P: I see. But I am a busy business person with a high stress job and travel interstate frequently. How could I make effective changes to my lifestyle? I think it is too difficult for me.
N: I see. No worries. I can understand why you think it is difficult. Many people in your same situation think it is difficult at first .but I will give you some practicable suggestions. Firstly, about

your diet I know you often eat out at regular places with clients but I am sure there are many choices available in restaurants. So make sure you order more vegetables, especially green ones, more fruits, and only a little meat. At that time, you can order fruit juice instead of alcohol. If you feel difficult at first, chose wine instead of beer and alcohol. I know you travel a lot so you do not have time for exercise but exercise can be any type. It can be as simply as having a walk around your house for 5 to 10 minutes every day. Even a small amount of exercise everyday will benefit you a lot. Even you do exercise when you are watching TV or when you drive to work, you can park your car a little bit far from your company and then walk to your office. Exercise will help you to lose weight. You know you should reduce your cigarettes, it is good. There is a quitting program in our hospital. I can contact them for you. They have many useful ways to help people to give up smoking. They are a wonderful help. About stress, everyone suffers from stress. You need to know some ways to relax. Say listening to soft music, developing your hobbies, join in a club. Your wife cares you, she gives you support so you do not do these alone.

11. Colon cancer

Setting: hospital ward

Patient: You are a 70 year-old patient who has had a bowel resection for colon cancer. Upon your recovery from surgery, the doctor wants you to start chemotherapy and them radiotherapy. Your father and brother both died of cancer and you are worried that no matter what you do, the cancer will return.

Task: 1. ask the nurse about chemotherapy and radiotherapy. Express your concern about side effects such as weakness and nausea, and hair loss.
2. Admit your worries to the nurse about managing alone at home.
3. Ask the nurse about the possibility of the cancer returning.

Nurse: The patient is a 70 year-old who has had a bowel resection for colon cancer. He was obese, but lost a lot of weight in the last few months. Since her hospitalization, she has given up smoking. Upon recovery from surgery, she is due to stat chemotherapy and then radiotherapy.

Task: 1. answer the patient's questions and give reassurance about the cancer treatment.
2. Reassure the patient she will be well looked after and explain that after she leaves hospital you will ensure support is available to her.
3. Answer the patient's questions in a reassuring manner.
4. make sure the patient has no further concerns to be dealt with.

N: Hello, Mrs. Smith, good afternoon. I am a registered nurse .I will be looking after you this afternoon. You have made a great recovery, so you will start chemotherapy and then radiotherapy. How are you feeling now?
P: I am worried about chemotherapy and radiotherapy. I heard they have side effects such as

weakness and nausea and hair loss as well.
N: I see. Many people having chemotherapy and radiotherapy feel the same way but no worries. They have some side effects like you said but these side effects are not serious, just minor. The doctor has prescribed some medication to reduce these side effects. We do have many ways to decrease the impacts of these side effects. For example, eating small frequent meals to prevent nausea, and increasing protein intake to prevent weakness and putting a cold pack over your head to prevent hair loss. Things like that. Besides, the chemotherapy and radiotherapy just last a short term. When they finish, these side effects would stop, I am sure you will be feeling better in a few days after they finish and you could go back to your normal life. We are experienced doctors and nurses. You will be looked after very well. If you feel uncomfortable, just let us know. We will manage any discomfort and make sure you will feel better. Besides, we do observations many times a day. Before you go home, the doctor will write a prescription for any medication for you. We could get the prescription filled for you so you would not have to go to the chemist on the way home. There might be some changes for your medication. We will also write a list of what you need and when to take it. I will make sure you have the list before you go home.
Do you live alone? Have you got any family or friends to help you? OK, I will arrange a home nurse to visit you and supervise your medication daily, and a carer to help with housework, like cooking, bathing, cleaning, dressing and she will also do the shopping for you, and chat with you. I will give the social worker's phone number. If you need anything else once you are discharged, you can put services in place.
P: I am worried about the cancer will return. My father and brother both died of cancer. I think no matter what I do it will return.
N: no worries. Most people in your same situation feel the same way but there are many factors to cause colon cancer. Family history is only one of them. We still can do a lot of things to prevent it. You have lost a lot of weight in the last few months. It is good .you need a healthy diet and continues to lose weight. You have given up smoking. It is great. Well done! Do you drink? I suggest you to give up drinking. Alcohol contains high levels of sugar and has no value of nutrition and damages blood vessels in the body. You need to do exercise regularly. Even a very small amount of exercise every day will benefit you a lot. Say having a walk around your house for 5 to 10 minutes every day or doing exercise even when you are watching TV. Be positive. Look on the bright side. Be happy. Join in a club, make new friends, that all can help you to prevent it.
Chemotherapy and radiotherapy reduce the chance of cancer coming back. You will have regular blood tests, combined with CT scans to minimize the chance of cancer coming back. Is there anything also you would like to ask or discuss? Please ask any question. If I do not know the answer, I will find out for you. I have some booklets about colon cancer. If you know a little more about it, I hope you would not feel so worried. If you are interested in them, I will bring them for you. Now I leave you to have a rest. Here is the bell, if you need me. Please do not hesitate, just give me a buzz. Remember we are always here for help. I will be back soon with the booklets. See you later.

(Chemotherapy is the treatment with drugs that kill cancer cells. Radiotherapy is the treatment with x-rays that kill cancer cells. They reduce the chance of cancer coming back.)

You do not have to worry a lot. With the right treatment, as long as you remember to take your pills and visit your doctor regularly, you will be ok and live a normal life.

There is an association for people with colon cancer. They have a lot of activities and they help people with colon cancer. Do you want me to refer you to them? You can meet other people with colon cancer. They all live a perfectly normal life. So you would not feel so worried.

12. Stroke

Setting: hospital wad

Patient: You are a 63-year-old truck driver and arrived in emergency via ambulance 2 days ago, after you experienced dizziness, vomiting and severe pain in your right ear, which radiated the right side of your face. All your symptoms came on suddenly while watching TV. You have been told you had a stroke and are currently taking medication. You are due to go home today.

Task: 1. tell the nurse you do not really know anything about strokes or what causes them. Express your surprise at finding out you had a stroke.
2. Show anxiety as you have heard people can die from stokes. Ask the nurse whether you will have another stroke in the future and whether you could die.
3. Ask about other measures besides medication which could assist in preventing another stroke.

Nurse: The patient is a 63 year-old truck driver who arrived in emergency by ambulance; his CT scan showed he had a stroke. He was stabilized in hospital with medication. He is due to go home today. He is to be treated long term with warfarin 6mg/day and is required to have regular blood tests.

Task: 1. ask the patient what they know about strokes.
2. Explain what causes strokes and what the medication is for (i.e. prevention and treatment of thrombosis).
3. Reassure the patient that her current condition is stable. Emphasize the patient of the doctor's orders to have regular blood tests, which will ensure he is closely monitored in order to prevent another stroke.
4. Advise the patient of preventive measures such as losing weight.

N: Hello, Mr. Smith. Good afternoon. I am a registered nurse. I will be looking after you this afternoon. Your diagnosis is stroke. Now you are stable and today you will go home. I thought it might be a good idea to talk about your discharge arrangements. What do you know about strokes?

P: I am surprised at finding out me have a stroke. I know little about it and what causes it.

N: No worries. Let me explain this disease for you. Stroke is a clot blocks a blood vessel in the brain. So people with a stroke have different symptoms according to the position of the clot in the brain. There are many risk factors that can cause a stroke, such as hypertension, diabetes, obesity, smoking, drinking, and unhealthy diet, lack of exercise as well. Higher levels of blood fats

make blood more easily to form clots. Many effective medications are available to prevent thrombosis, for example, Warfarin. Your situation is stable with Warfarin. You need to take it for a long term. The doctor has ordered regular blood tests for you which will make sure that you are monitored closely to prevent another stroke.

P: I have heard people can die form strokes. Whether will I have another stroke in the future and whether could I die?

N: No worries. The rate of dying from strokes is really low. The rate of having a recurrent stroke is very low and now you are taking Warfarin to prevent another stroke. The drug is effective. Many people with a stroke have taken the medication. Most of them do not have a recurrent stroke but remember having regular blood tests is very important, is the key to prevent a future stroke. They will monitor you closely in the future.

The doctor will write a prescription for any medication for you. We can get the prescription filled for you so you would not have to go to the chemist on the way home. There might be some changes for your medication. We will write a list of what you need and when to take it. Warfarin is 6 mg / day. I will make sure you have the list before you go home. As you know, besides medication, there are many other ways of prevention. Give up smoking and drinking. Have a healthy diet, lose weight, and do exercise regularly. They will benefit you a lot.

There are some warning signs we need you to be aware, if you could not speak clearly, fluently as normal, could see things clearly, have a sudden headache, have numbness on one side of the face or the body. I need to contact your doctor immediately. OK?

Is there anything else you would like to ask or discuss? Please ask any question, if I do not know the answer, I will find out for you. I have some booklet about strokes, if you know a little more about what it is, what to expect, what help is available, I hope you would not feel so worried. If you are interested in them, I will bring them for you. Now I leave you to have a rest. Here is the bell, if you need me, please do not hesitate to give me a buzz. Remember we are always here for help. I will be back soon with the booklets. See you later.

Stroke

A blood vessel carrying blood to the brain is blocked by a blood clot. Higher levels of blood fats make blood more easily to form clots. Fatty deposits collect on the wall of the arteries. Over time, the deposit builds up and blocks the blood flow.

13. Injury

Setting: accident and emergency

Patient: You are a 21 year old who sustained severe burns to your right arm after a traffic accident. You are currently in chronic pain. You are to have a skin graft on your arm. Some months ago, you moved out of your parents' home to a flat with 3 friends and your present lifestyle is hectic and full. You are anxious and worried that your lifestyle will change. You are deeply concerned that you may have lost full use of your arm.

Task: 1. Ask the nurse what is going to happen when the skin is grafted. Will it heal? Will you have the full use of your arm?

2. Express worry about your new job and commitments to your basketball team and your need to resume those as soon as possible.
3. Ask if this kind of pain is normal? Could it mean the injury is permanent?

Nurse: The patient is a 21 year old who sustained severe burns to her arm after a traffic accident. She is currently in pain. The patient has been told she will need to have a skin graft.
Task: 1. Answer the patient's questions in a calm manner.
2. Reassure the patient and offer to arrange a friend or relative to be present.
3. Explain to the patient that although burn injuries feel painful, she is readily treatable and it is normal for people to successfully resume their normal lifestyle.

N: Hello, Miss. Smith. Good afternoon. I am a registered nurse. I will be looking after you this afternoon. How are you feeling now?
P: I have got a pain in my right arm. I will have a skin graft. What is going to happen when I am having it? Will you have the full use of my arm?
N: No worries. Burns usually cause pain. It is normal. During the skin graft, the doctor will take a part of skin from your leg or your back and then stitch it to the burnt part of your right arm. You will be feeling better in a few days later, it will heal and the pain will disappear. You will have a full recovery, means you can use the arm as normal as before.

P: I am worried about my new job and want back to basketball team and begin as soon as possible.

N: No need to worry. Only your skin is burnt. Your muscles and bones in your right arm have not been harmed. So their function is normal. You just need some time to recover after the skin graft. When the skin heals, you will have the full use of your arm and go back to your normal life. No problems with your new job. You also can go back your basketball term. Just take time. let the wound heal. Does your family know you are here? Do I need to contact anyone for you?

P: I am in chronic pain. Is it normal? Could it mean the injury is permanent?
N: It is normal; burns always cause pain, especially skin burns. Because, skin contains all of the nerve endings. That is why you feel so much pain. It does not mean the injury is permanent. After the skin graft, the pain will disappear. When did you take painkiller last time? Ok, I will have a talk with the doctor and let him know you are suffering serious pain. Maybe you can have another dose of painkiller. I am sure after the skin graft; you could go back to your normal life.

(A skin graft is the removal and transplantation of healthy skin form one are of the body to another area where the skin has been damaged. The source sites most commonly used for skin grafts are the inner thigh, leg, buttocks, upper arm and forearm.

Post-op
Keep the wound clean and dry. Avoid all traumas to the wound. Do not expose wound to prolonged sunlight. Inspect site for healing and good circulation, as shown by healthy pink coloration.
Follow instruction given for bandaging the grafted area to provide it with appropriate support during the healing process and to prevent contracture even after healing is completed. Use a plain ole product to lubricate the recipient site, since there are no functioning sweats or oil gland in a skin

graft. No smoking. It slows or stops the healing by slowing oxygen and blood supply to the grafted area. Eat a healthy diet with plenty of protein intake.

Call GP if any of the following occurs
Signs of infection; including fever and chills. Redness, swelling, increasing pain, excessive bleeding, or discharge from the incision site. Headache, muscle aches, dizziness or general ill feeling.
Cough shortness of breath, chest pain or severe nausea or vomiting.)

16. Bed wetting

Setting: women's and children's hospital

Patient: Nicholas grey, 10 years, has been referred to the bed wetting clinic by the local GP. He is embarrassed and reluctant to talk about his problem.

Nurse:
Task: 1. reassure him that his problem is a common one and that can be fixed.
2. Ask him how frequently he wets the bed, how much he drinks and when and what his parents have done so for to help him.
3. Assure him that it is not his fault. Explain what the causes may be.
4. Suggest that he does a chart of nocturia frequency along with using a nasal spray. Explain how it is used. Say you will see him in a fortnight.

N: 1. Millions of kids and teenagers from every part of the world wet the bed every single night. It's so common that there are probably other kids in your class who do it. They just don't tell their friends, so it's easy to feel kind of alone, like you might be the only one in the whole world who wets the bed. That is not true. you are not alone. Actually in this clinic, we have helped a lot of children with the problem.

2. This problem can be fixed. We can help you. But firstly, we have to know you more. I'd like to ask you a few questions. You have to be honest. Ok?

How often do you have the problem, say, once a week, twice a week, or once a month?
How much drink on average do you have daytime?
When did your parents know you have this problem?
What have they done so far to help you?

3. Having the problem is not your fault. It runs in families. In other words, there's a good chance that a close relative also did it when he or she was a kid. Just like you may have inherited your mom's blue eyes or your uncle's long legs, you probably inherited this problem from one of your close relatives.

I know no one wets the bed on purpose. You need to remember this. It is not your fault. It doesn't mean that you're lazy. You just are not able to feel that your bladder is full when you are sleeping and don't wake up to pee in the toilet.

4. The good news is that almost all kids who wet the bed eventually stop. Don’t worry that you're not normal, either physically or emotionally. It's likely that bedwetting will go away on its own even without any treatment at all. There are some things you can do to stay dry. Try not to drink anything after dinner and remember to go to the bathroom before going to bed. If you do wet the bed, help with the cleanup by pulling off the sheets and putting them in the laundry.

A bedwetting alarm would help you a lot. The alarm is connected to a pad placed in your underwear at night. If you start to urinate, the pad senses the moisture and sets off the alarm. Different alarms buzz, vibrate, or do both, but they're all easy to use and can help wake you up. I will have a word with your parents about the alarm. I’d like you to keep a dairy, record the date that you wet the bed. You can draw a chart on your own, or the next time that I see you we can do the chart thing together. I will see you 2 weeks later.

Your doctor has prescribed a nasal spray for you. It is easy to use. I will show you. You need to use it everyday before you go to bed. It reduces urine production during sleep. Hold it in front of your nose; make sure the nozzle toward your nose. When you are ready, press the button on the top, take a deep breath in, hold your breath a few seconds, then breathe out.

It takes time. You’ll need to be patient. But eventually, you will stop wetting the bed. I will see u 2 weeks later and see how you go.

17. Multiple fractures

Setting: Modbury hospital

Patient: Brad Wilins, 23, has just been urgently admitted to hospital after an accident in a mine. He has multiple fractures (legs, ribs) and is unconscious.

Nurse:

Task: 1. ask his friend, Marco Bruni, who has accompanied him in the helicopter, about the circumstances.
2. Explain the probable procedures he will undergo.
3. Ask about Brad’s general health and contacting his family.
4. Reassure friend.

N: 1. I am the nurse who will be looking after him today. I am so sorry about the accident. I can see you are very worried about him. I can understand. Most people in your same situation feel the same way but do not worry. Now he is in hospital. He is in good hands. We are providing the best possible treatment and nursing care for him. Experienced nurses and doctors are stabilizing his condition. In the meantime, I would like to ask you a few questions. They are important for the treatment and care. I know you are the one who has been with him in the helicopter. We would like to know more about how the accident happened and what first aid he got before the paramedical staff arrived. Please recall the accident. We need details. Give us as much information as you can.

2. Doctors and nurses are stabilizing his condition. His vital signs and conscious state are closely monitored. We suspect he has multiple fractures, so he is having examinations and tests like x-rays, blood tests and urine analysis to determine the extent of the injury. Once we get the results of those tests, we will have a clearer idea of his condition. We keep him fasting for the moment, just in case he needs to go to the theater.
3. We would like to know more information about his general heath. Do you mind if I ask you more questions? What does he do? Does he smoke or drink? Did he smoke or drink? How many cigarettes on average do he smoke everyday? Have long has he smoked? What about his diet? Does he eat everything? Has he had any serious illnesses in the past? Has he had any operation in the past? Is he allergic to anything you know of? Did he have a bad reaction or a rash after taking it? Does he have any history of serious illnesses in his family? Does he take any medications regularly?
Does his family know he is here? We really need to contact his family as soon as possible. Have you got the contact numbers of any his family members? I will get on to that as soon as we have finished here. Thanks.

4. No worries. Have a drink. I will go and check his condition. I will be back in a moment and tell you his condition.

19. Heart attack

Setting: hospital ward

Patient: You are in hospital after collapsing at work with a mild heart attack. You have only been in hospital a couple of days and already are having lots of visitors and much excitement. You love it; you think you thrive on this sort of stimulation.

Task: You want to keep up this pace despite the objections of the nurse.

Nurse: The patient has been admitted after collapsing at work with a mild heart attack. Now, after being in hospital, only a couple of days, she is getting too many visitors and too much excitement.

Task: Suggest ways to limit the visits and to make the patient get some rest.

N: Hello, my name is Dae, I am a RN. I will be looking after you this afternoon. How are you feeling now? I am glad you are feeling better. You are making good progress. Your condition is stable now. You are on the way to recover. In this stage, having plenty of rest is still important. It would be a good idea to take afternoon naps. We would like you to get rest as much as you can. The more rest you get, the quicker you will be able to go back your normal life. I know it is no fun in hospital. Everyone wants to go home as soon as possible. We are all hoping you get rest as much as possible and recover quickly. So we would like you to have an afternoon nap on most if not all days during your hospital stay? Do you think you can do that?

We notice you have got a lot of visitors. Are they all your family and friends? Wonderful! You must get a lot of support from them.

I have got an idea. Would you like to have dinner with them? If we arrange them to visit you around dinner time, you can have dinner together and then there would be plenty of time for a chat before you go to bed. The important thing is you would have time to take an afternoon nap. Does it sound OK?

(I know you got a collapse at work due to a mild heart attack. I am sorry about that. You have been in the hospital for a couple of days. During the period of time, you have made a great recovery and now your situation is stable. So I would like to talk to you about heart attack. Do you know anything about heart attack?

Let me explain it. First of all, a heart attack occurs when the blood supply to the heart muscle is reduced significantly or unfortunately stopped completely. In other words, a heart attack occurs when the blood supply to the heart muscle is not able to meet the heart muscle's needs.
Many factors can trigger this situation, like a heavy, large meal, excessive alcohol intake, a serious flu, poor quality of sleep, stress, strong emotional swings. Because those things make heartbeat faster, make the heart muscle consume more oxygen. You know, the blood contains oxygen. So the blood supply to the heart muscle is also the oxygen supply to the heart muscle. For example, when a person is excited, his heart needs more blood supply and more oxygen than it does when the person is calm. In this situation, if his body could not provide extra blood supply and oxygen to his heart due to any reason, there would be more likely to occur a heart attack.

When we are sleeping, when we have a rest or when we feel calm, the heartbeat is very slow, the heart muscle needs a very small amount of blood supply and oxygen. That is why rest, good quality of sleep and calmness is so important for patients who have some problems with their heart.

We have noticed during the couple of days when you are here, you have got a lot of visitors and when you are with them, you look excited. Is it true? We are glad you have got so much support from them. We know they are supportive. They all care about you. However, we do not think those visits are good for you at this stage, because they took time and you got excited.

We want you to be calm. We want you to have enough rest. We want those things because we hope you could recover as soon as possible and we want to put you on the safe side and then go back to your normal life.
We know your friends want to come and see you and express their care. Sometimes it is very difficult to say no to them. Have you got any family who can do that for you? Or we are here for help. If you want, we can do that for you.)

20. High Blood Pressure

Setting: medical centre

Patient: You are brian, a 35 year old truck driver. You have been suffering from high blood pressure for a few months. You are overweight at 105 kg for a height of 175 cm. You smoke 20 a day and you eat fast food. In addition you do not do any exercise.

Task: you want to become healthier but you have no willpower. You work long hours which makes it difficult to exercise regularly. Find out how you can lead a healthier lifestyle. You are reluctant to change too much though.

Nurse: You are talking to a 35 year old man who has come in today suffering from high blood pressure. He is overweight, he smokes a pack of cigarettes a day and he eats a lot of fast food. In addition he does no exercise.

Task: Find out as much as you can about the man's lifestyle.
Advise him on how he can lead a healthier life.
Warn him of the dangers of his present lifestyle.

N: Hello, my name is Dae, I am a RN. I will be looking after you this afternoon. You are suffering from hypertension, aren't you? High blood pressure is associated with some lifestyle factors. So I would like to ask you a few questions. Is that OK?

1. Collect information.

Do you smoke? How many cigarettes do you smoke on average every day? How long have you smoked?
Do you drink beer, wine or alcohol? How many glasses do you drink on average every day? How long have you drunk?
What about your diet? Do you eat everything? How about fruit or vegetables? Do you cook?

Do you live alone? Do you have a family? Is there someone who can cook for you?

Do you doing exercise? Occasionally, or regularly?

What is your hobby? What physical activities do you like?

What is your job? So you work overtime often, don't you?

Tell me about your sleeping habits? Do you take anything to help you sleep?

What do you usually do when you feel stressed?

How long have you been suffering from hypertension? Only a few months. I see. This is a very early stage of hypertension. In this stage, BP can be managed quite effectively by the combination of medications and lifestyle modifications. It sounds horrified? No. we just need some little changes to your life.

2. Smoking

Chemicals in cigarettes damage the blood vessels. Cigarette smoke forces arteries to constrict. So try to stop smoking. I know giving up smoking seems impossible to you now but, if you take it gradually, and have the support of your family, you will be able to reduce you dependency and improve your health considerably. I can also put you in contact with the Quit line organization today. They are very successful in helping people like you give up smoking permanently.

3. Diet

I know you don't cook and there is no one who can cook for you. You work long hours so you eat fast food. Remember not all fast food is junk food, like salad sandwich and sushi. You just need to explore more healthy options. Have a serve of fresh fruit with your meal. There are a lot of different choices. Eat more fruit and green vegetables. Choose sandwiches that contain whole meal or whole grain bread. Speaking of eating habits, avoid large meals, have frequent small meals.

4. Drinking

Alcohol also damages the blood vessels, so I'd recommend you cut down on alcohol. Drink skim milk or sugar free fruit juice instead. Remember water is always the best. You are still allowed to enjoy life. You still can have a little. Just do not go overboard.

5. Stress management

What do you usually do when you feel stressed? When you feel stressed, your BP always goes up. When you feel stressed, do them to relax. No, that is no good. You need to find other activities to relax. Just try something different, like reading, watching movies, listening to music. You will find something that can relax yourself. If you have difficulty finding something, contact me any time. I can recommend you some stress management programs, where trained instructors teach people stress management strategies.

6. Exercise

I know regular exercise is impossible to you. But you would benefit from any amounts of exercise, even if you do them while you are watch TV or something. It is nerve late to start exercise. Add exercise to your life routine. For example, cleaning, gardening and walking to the supermarket.

7. Weight management

Another important thing to do is to keep a healthy weight for your height. There are a number of options. You can do exercise in your free time to control your weight, or join a weight management program or I can refer you to a physiotherapist. Think about those options, ok?

8. Harm

Your present lifestyle is strongly linked to your high BP. Without lifestyle modifications, medication only can not manage it effectively; therefore, the dosage of medication will have to keep increasing. You know, every drug has side effects. We want to keep your BP under control with the smallest effective amount of medication. Over time ill managed hypertension can lead to a number of complications. So we'd like you to make some little changes to your life, hopefully, with the medication, we can keep your BP under control.

21. Hip replacement

Setting: hospital ward

Patient: you are an 80 year old woman who has had a total hip replacement following a fall in your home. Prior to your fall, you lived independently with your husband. You are about to discharged to a rehabilitation hospital. Although your recovery has been good you are feeling very anxious about further falls, your mobility and your ability to resume your previous life.

Task: Seek reassurance about your condition including mobility and the possibility of further falls.
Ask about help available to you when you return home.
Explain that your 82 year old husband has a cardiac condition and your only child lives overseas. Be difficult to reassure.

Nurse: The patient is an 80 year old woman who has had a total hip replacement following a fall in her home. She is about to be transferred to a rehabilitation hospital. Although the patient has made a good post operative recovery, she appears very apprehensive and agitated.

Task: Find out about he patient's concerns.
Reassure the patient about her prospects of resuming a worthwhile life following her rehabilitation.
Inform the patient of services and help available to her on her return home.

N: I am a RN. I will be looking after you this afternoon. I am glad you have made such a great recovery since the operation. You are going to the rehabilitation hospital. Congratulations! How are you feeling now?
I see. I can understand what you are feeling now. Most people in your same situation feel the same way. That is why we refer you to the rehabilitation hospital, instead send you home directly. No worries. In the rehabilitation hospital, there are nursing staff who will be looking after you 24 hour a day and therapists who will give professional instructions to help you improve your muscle strength and joint flexibility. That is usually just for a short time and will enable you to be more confident and independent when you return home. For example, the physical therapist will work on mobility, strengthening, and walking. The occupational therapist will work with you on preparing for tasks such as washing, dressing, and other daily activities.
Do you live alone? Have you got any family or friends to help you?

I know it must be really hard for you to look after your husband, especially he has a cardiac condition and your child lives overseas but don't worry. We are here for help. There are a number of community services available to help you. You would not have to do all the housework and look after your husband by yourself. Like meals on wheals, a care.
If you agree, we will refer you to the Aged Care Assessment Team. They will send someone to the rehabilitation hospital and assess you condition. According to your condition and needs, they may recommend a Community Aged Care Package. That would be very helpful for you. And social

workers in the hospital may be able to help you to find a package. The package may include a personal carer. The carer would come each morning to assist you to shower and dress. The carer can also help you with housework, like washing, cleaning and so on.

We may refer you to domiciliary care. They will send someone to your home to assess your house and give you some suggestions about how to improve safety and encourage independence for you. For example, they may suggest you to install handrails next the toilet and in the shower or suggest you remove rugs in the house. The domiciliary care handyman can carry out these modifications in the home. They will advice you what equipment you may need when you go home, such as a toilet seat raiser or a shower chair. These equipment can be loaned for a small cost from the domiciliary care. I will give you the contact number of Independent Living Centre, which is a government organization where people can borrow equipment for relatively low fees. Just in case, the equipment you need is not available in the domiciliary care. You know, they are very popular.

22. Acute Lymphoblastic Leukaemia

Setting: a hospital ward

Patient: You are Joan, whose daughter Marry has just been diagnosed with ALL. You are very upset and worried about your daughter's condition.

Task: try and explain how you feel to the nurse.
Ask about possible treatment for your daughter.

Nurse: You are talking to Joan whose 4-y-o daughter Marry has UST been diagnosed with Acute Lymphoblastic Leukaemia(ALL). Joan is very upset about this and she is very worried about her daughter.

Task: Listen to Joan's worries.
Try and reassure her about her daughter.
Explain what possible treatment will be given to her daughter.

N: I am a RN, who will be looking after your daughter this afternoon. How are you feeling now?
I can see you are worried about your daughter. I am so sorry about her. I know it is hard for you to take. You must be scared and confused. But don't worry. There are a wide range of treatment options available. Science is making amazing progress in this area. There are new drugs and new therapies. She is in hospital. She is in good hands. Her doctor is an experienced specialist. There are professional nursing staffs that are looking after her. We will support her and care for her in the best possible way. We will try our best to take good care of her and make her feel comfortable. I know you will support her. This will make all the difference. Would you like a talk? If not, it is ok. You can contact my any time with any concerns you have. I will be here for you.

Once you know a little more about the disease, what to expect and what help is available, you would not feel quite so worried. So I thought it may be a good idea to talk with you about what the

disease is briefly first.
Acute Lymphoblastic Leukemia is a cancer of the white blood cells. Lymophocytes are a type of white blood cell that the body uses to fight infections. In acute lymphoblastic leukemia, the bone marrow makes a lot of abnormal lymphocytes. They can not function normally; therefore they can not fight infections. The number of abnormal lymphocytes grows quickly. They crowd out the normal red blood cells and white blood cells. That is why people with ALL look pale, have fatigue and they are prone to infections.
Basically, there are three treatment options: chemotherapy, radiotherapy and bone marrow transplant.

Chemotherapy is the use of drugs that destroy cancer cells or stop them from growing. Chemotherapeutic drugs may be taken as tablets or given by intravenous drip directly into a vein.

Coping with side effects; drink plenty of water in the days after your chemo treatment. Exercise gently whenever possible. Take afternoon naps if necessary. Eat a healthy diet with plenty of fresh fruit, veggies and lean meats. Avoid large meals, spicy or greasy foods.

Radiotherapy is the use of precisely targeted x-rays to destroy cancer cells. Normal cells can recover from radiotherapy. The length of treatment varies, depending on individual factors, such as patient's age, general health, the location, type and stage of the cancer.

It is usually given once a day, 5 times a week and takes a few minutes each time and it is painless. It is also used after surgery to kill off any remaining cancer cells.

Not every patient experiences side effects, usually 1 or 2 side effects occur. Side effects depend on the type and dose of radiotherapy patient receives and which part of the body is being treated, including fatigue, nausea, loss of appetite, skin changes (dryness, reddening, itching, blistering, flaking, tanning and superficial ulceration, which will heal in 2 to 6 weeks.) hair loss, mouth issues (mouth dryness, difficulty chewing and swallowing and dental decay), chest problems (coughing, SOB, painful swallowing). It is important to remember that almost all side effects will disappear once treatment is completed.
Bone marrow transplant offers some patients the best chance for a long term remission of their disease.

Every therapy has side effects. But not every patient experiences side effects. Usually one or two side effects occur. And it is important to remember that almost all side effects will disappear once treatment is completed. We do have effective strategies to prevent some side effects from happening or reduce the severity of side effects. For example, nausea and fatigue are the most common side effects. To prevent nausea, we will offer her small amount of meals frequently. To reduce the level of fatigue, it would be a good idea to take afternoon naps. We will encourage her to take a nap in the afternoon on most if not all days during her hospital stay and make sure her room is quiet and suitable for a nap.

Her doctor will order more tests to identify what type of ALL she has and then talk you through the possible treatment options. He will talk about advantages and disadvantages, benefits and risks of all the possible treatment options, then you both will make a decision about what treatment is best

for your daughter.

Leukemia foundation in SA is an wonderful organization. They have a variety of services. They help people with leukemia, their family and friends. They provide information, help and support. They provide opportunity for people with leukemia, their family and friends to meet, exchange tips and experiences. I can put you in touch with them and I will give you their website.

When she goes home, you need to make sure she takes her medications as ordered, and make sure she has regular medical checkups. There are some things we need you to be aware of. If you notice she looks pale or she is easier to get tiered than normal, you need to contact her doctor immediately. Next if she has a fever or any kind of infections, you also need to call her doctor immediately. Have you got his contact number? No worries. I will make sure you have his contact number before you and your daughter leave hospital. Before she leaves hospital, her doctor will write a prescription for any medication for her. There may be more than one medication. We will write a list of what she needs and when to take them.
With treatment, your daughter would go back normal life and be a healthy girl.

24. Rheumatoid arthritis

Setting: a medical centre

Patient: You are a 50 year patient who suffers form rheumatoid arthritis. You were diagnosed with this condition about a year ago and it has got steadily worse since then. You feel angry and frustrated with your condition. You used to enjoy walking and horse riding before. Now you fell kike an invalid.

Task: Tell the nurse how bad you feel.
You desperately need help. Ask the nurse how you can improve the quality of your life. You feel very depressed.

Nurse: You are talking to a 50 year old woman who suffers form rheumatoid arthritis. She was diagnosed with this condition about a year ago and it has got worse since then. She is very frustrated with her condition as she used to be a very active woman.

Task: Find out how her condition affects her both mentally and physically.
Try to give her some advice on how she can improve her quality of life.
Try and be positive with the patient.

N: How are you today? I am sorry about that. How long have you been suffering from rheumatic arthritis? What symptoms have you been experiencing? Have you had any problems with your daily activities? How have you been feeling with those symptoms?

1. (Medications) Has your doctor prescribed any tablets for you to control your rheumatic arthritis? Have you taken them as ordered? Yes, that is good. Have you had regular checkups with your

doctor?
2. (Exercise and rest) Do you exercise regularly? It is important to get plenty of rest. But do not spend all your time in bed or resting. Keep as active as you can. Do normal daily activities. You need to balance rest and activity. Do regular exercise. Exercise can help reduce symptoms, ease pain and increase your flexibility. I can refer you to a physiotherapist who will give you professional instructions to help you improve your muscle strength, joint mobility, flexibility and balance. It would be an easy and safe way to get exercise started.
3. (Stress and relaxation) What do you do when you feel stressed? That is good. That is stress management. Keep doing them. When you feel stressed, do something to relax. During periods of time when you feel stressed, your symptoms may worsen. I am sorry about that. You need to find something else to relax. Try something new, different, something you are interested in. if you are not sure whether they are safe for you, you can seek advice for your doctor. If you can find something, I would recommend you some stress management programs.

4. (Diet) Your diet affects your overall health so having a healthy diet is important. Like a diet low in fats and calories and with plenty of fresh fruit and green vegetables.

5. (Alternatives) There are some complementary therapies that may be helpful, like massage, acupuncture. You can consult them with your doctor.

6. (Support) you know, arthritis SA is an wonderful organization. They have a variety of services. They help people with arthritis, their family and friends. It is always better when you know where to get information, help and support. They provide opportunities for people with arthritis to meet, share and exchange information, tips and experiences. I can put you in contact with the organization. They are very successful in helping people with arthritis like you live normal lives.

7. (Attitude)

(Rheumatoid arthritis (RA) is a form of arthritis that causes pain, swelling, stiffness and loss of function in your joints. It can affect any joint but is common in the wrist and fingers.)

26. Immunization

Setting: a medical center

Patient: You are Christine, a 35 year old mother of a 2 month old baby boy Johnny. Johnny will have his 2 month immunizations next week. You are worried about this as you have heard some terrible things about these injections.

Task: Try and find out as much as you can about these immunizations.
You are worried your son will be damaged by this procedure. You do not want to do it.

Nurse: You are speaking to Christine, a 35 year old mother who has a 2 month old baby boy

Johnny. Johnny will have his 2 month immunizations next week. Christine is very worried about this.

Task: Explain to Christine why her son needs to be immunized.
Reassure her as to the procedure and the possible side effects.
Try to make the mother feel good about what will happen to her baby.

N: (Reasons) In the first months of life, a baby is protected from some infectious diseases by antibodies transferred from mother to baby during pregnancy. When these antibodies wear off, the baby is at risk of serious infections. Having your child immunized helps protect them against the most serious childhood infections, some of which may be life-threatening, like hepatitis B, chickenpox, tetanus, measles and mumps. So immunization from an early age is highly recommended for all Australian children. Prevention is always better than cure.

(2 month immunizations) Diphtheria. Tetanus. Hepatitis B.

(Side effects) Mild temperature, Drowsiness, Irritability/crying; These side effects are generally mild, occur soon after the immunization and last for only a short period. Treatment is not usually required.

(Procedure) Most childhood immunizations are given as an injection in the arm or leg. The whole procedure only takes around 5 minutes. But it is better to remain at the clinic for at least 15 minutes after the child is immunized just make sure there are not adverse effects.

(Safety and effectiveness) Some parents are concerned about vaccines' safety and effectiveness. Every vaccine used in Australia was thoroughly tested for safety and effectiveness. Vaccine preparations contain dead or weakened organisms that do not cause infection. They may have side effects, but they are mild and the risk of complications from childhood disease is much higher than the risk of immunization. So having your baby immunized is the easiest and best way to prevent them from many serious childhood infections.

27. Blood test

Setting: Flinders medical centre

Nurse: You must take a blood sample from a 31 year old man with high blood pressure.

Task: Ask about the patient's reason for referral
Explain 3 features of a blood sample which may give a clearer idea of the patient's condition.
Reassure the patient about possibility of treating his likely condition, whatever it is.

N: I will be looking after you this afternoon. Could you tell me what's brought you into hospital today? How long have you been suffering from HT? Have you had any other symptoms?

The doctor has ordered a blood test for you so I have to take a blood sample from you. You will have a little pain when I do that. But if we do not know what's causing the problem, we can not treat it effectively. HBP can be a symptom of many medical conditions. Once we get the results back, we will have a clearer idea of your condition.
There are some possibilities. For example, the blood test can tell us your blood cholesterol level. If it is higher than normal level, it means more cholesterol than normal in your blood circulation. In this situation, cholesterol is easy to deposit on the lining of the blood vessels, which causes hardening of the blood vessels. Once blood vessels lose their elasticity to some extent, the blood pressure will go up. In this case you may need cholesterol lowering medications to keep it under control and some lifestyle modifications.

Next, the test can tell us your blood electrolytes levels. Blood electrolytes are salts in the blood. The kidney is a major organ to balance blood electrolytes. So if their levels are abnormal, we will suspect some problems with the kidney. So the doctor may order further tests to determine the problem. The treatment options depend on the type and severity of the problem.

Then, blood test can tell us your blood levels of certain hormones. There are a number of hormones that regulate BP. The abnormal levels may indicate some problems with the glands that release those hormones. In this case, you will need more tests or x-rays to determine which gland causes the problem. Treatment options depend on the nature of the problem. You may need medications or a surgery.

28. Fall in Nursing home

Setting: Nursing home

Patient: Mrs. Ivy Branson, 74, has recently become a resident in the Southern Cross nursing home at plympton, her husband died 2 years ago and she recently had a fall in the garden in the nursing home and broke her ankle. Since her accident, Ivy has lost all interest in life and refuses to leave her bed.

Nurse:
Task: Ask her about her recent fall.
Explain that it is not serious and should not prevent her form being mobile.
Question her about her lifestyle before she entered the nursing home.
Try to find out what kinds of things she is interested in.
Encourage her to participate in the various activities provided by the nursing home for the residents.

N: I am sorry about your fall. Could you tell me about the fall you had? How is your ankle going? Is it still painful? Can you walk independently? Are you still unsteady on your feet?

That is good. That means you have got a full recovery. You can get your normal life back.

I see. I thought you would benefit from more rehabilitation exercises. I can refer you to a physical

therapist who will work on mobility, strengthening, and walking; work with you on preparing for tasks such as washing, dressing, and other daily activities. This is just for a short time. And will enable you to be more independent and confident when you are walking. Does that sound good?

I know it has been a big change in your life to come here as a resident. Could you tell me a little bit of your life before you settle in here? How did you spend your time before? What's your hobby?

We organize a variety of activities. We have many groups such as knitting group, cards group and reading group. Why don't you come to the excursion next Thursday? Why don't you joint in knitting group? In the group, there are more than 10 residents. They all like knitting. They have regular activities every week. They knit together, they talk, exchange experiences and tips about knitting. They have much fun each time. I can introduce you to the group. What do you think?

Life continues. Fine yourself a new hobby or get involved in hobbies that allow you to interact with others. Hobbies are activities that you can find opportunity to interact with others. When you feel low or upset, you can at least turn to something that brings you comfort and enjoyment when doing it.

30. Dementia homecare

(Greeting and collect information) How are you? How is she going? Have you had any community services at home to help you? Have you got any family or friends to help you?

(Condition reviewed) I thought it would be a good idea to visit her GP for a full assessment of her condition and to have her medications reviewed by her GP. If her condition is progressing, if it would be unsafe to leave her living at home any more, we can seek advice from her GP about which place is suitable for her current condition, a nursing home or a hospital.

(Hospitalization) Whatever it is a nursing home or a hospital, there she would have 24 hour medical care and you can visit her and stay with her whenever she likes.
How long have you been married? It is a really long time. I can understand why you are feeling this way. You want to keep her home as long as possible. I understand.

(Staying at home) If her GP says her condition is ok, she can still live at home. There are a number of services available for help.
(Services)
(ACAT) We can refer her to the Aged Care Assessment Team. They will send someone to your home and assess her condition. According to her condition and needs, they may recommend a Community Aged Care Package. That would be very helpful for you. And social workers in the hospital may be able to help you to find a package.

(Carer) According to her condition, the package would include a personal carer. The carer would come each morning to assist her to shower and dress. The carer can also help you with housework, like washing, cleaning and so on.

(Meals) What about meals? Have you got any problems with preparing meals for you both? You could have meals on wheals.

(Medication) Have you had any problems with managing her medication? Can you tell me how you have managed her medication? Could you make sure she takes her medication as ordered? Is there any problems? We could ask her GP to send the prescription for all her medication to the pharmacist so that the pharmacist would prepare a Webster pack for her. Each Webster pack contains all the medication she needs for a whole week. There will be clear instructions on the pack with date and time that the medications need to be taken. The pharmacist may be able to deliver the pack to your home or you can pick it up each week. The Webster pack is only for tablets. If she needs injections, we will arrange a district nurse to visit and give her medication.

(Respite services) I know the constant demands of caring for a family member can be exhausting. It is also important to take good care of yourself. It is important to take some time out just for yourself. If you agree, I will refer you to the respite services. They would a good help for you. A community support worker from the service will visit you to assess your needs, and then you both will make a plan together about how they will help you and the times they will send someone to your home to look after her. It could be for a few hours, overnight or a couple of days as you wish so that you can have a break to relax and refresh.

(Support and education) I thought it is better when you know where to get support and information. Carers SA is a wonderful organization. They have a variety of services. They have carer education programs. They have carer support groups. They provide opportunity for carers to meet other carers to share and exchange experiences and tips. I can put you in contact with them. They are very successful in helping and supporting care-givers like you.